

External Partner Selection, Qualification and Oversight SOP

Loblolly Ridge Therapeutics | Quality Assurance with Outsourcing | 2026-01-15

SOP QA-014, revision 4.0. This procedure sets the requirements for selecting, qualifying, contracting, overseeing, requalifying and exiting external partners. It applies to every partner whose work supports a GxP activity or a GxP decision, and it scales the level of control to the partner's risk tier.

1. Purpose

Loblolly Ridge relies on partners for clinical operations, GLP studies, bioanalysis, manufacturing and packaging. Delegating the work does not delegate accountability. ICH Q10 makes the pharmaceutical company responsible for controlling outsourced activities, including assessing suppliers before use, putting written agreements in place and monitoring performance. For clinical trials, ICH E6(R3) allows a sponsor to transfer activities to service providers but keeps ultimate responsibility with the sponsor, and activities not specifically transferred stay with the sponsor.

2. Scope

All partners performing GCP, GLP, GMP or pharmacovigilance work for Loblolly Ridge, and partners whose systems hold GxP records. Non-GxP partners (tier 3) are covered by sections 5.1 and 5.6 only. Academic collaborations follow the research agreement and are outside this SOP unless the collaborator generates data for a regulatory submission.

3. Responsibilities

Program owner: defines the need, takes part in selection, reviews deliverables and leads quarterly oversight.

Outsourcing & Procurement: runs selection, maintains the partner register and contracts, and schedules oversight reviews.

Quality Assurance: assigns the risk tier, performs or commissions audits, approves qualification and owns quality agreements.

Legal: reviews master agreements, work orders and quality agreements.

Finance: reviews invoice accuracy for the oversight review.

4. Risk tiers

Tier 1 (GxP-critical): CROs, central laboratories, GLP testing facilities, bioanalytical laboratories, CDMOs and clinical supply partners. Audit before first use and every 24 months.

Tier 2: other GxP-relevant partners such as IRT, ambulatory blood-pressure monitoring, patient recruitment and regulatory publishing. Audit every 36 months or assessment by questionnaire.

Tier 3: partners with no GxP impact. No routine audit.

5.1 Selection

The program owner documents the need and the selection criteria. New services above \$250,000 need at least two proposals unless a single-source justification is approved by the Director, Outsourcing & Procurement. Quality Assurance confirms the proposed tier before a partner is shortlisted.

5.2 Qualification

Qualification requires a documented assessment, a quality agreement for GMP and GLP partners and for CROs, and QA approval before any GxP work order is signed. For a GLP testing facility, the assessment confirms that the facility has a quality assurance unit independent of the personnel conducting each study, as 21 CFR 58.35 requires.

QA may approve a partner as qualified with conditions. The condition has a date and an owner, and is usually an audit within 12 months.

5.3 Agreements

The quality agreement defines each party's quality responsibilities and is kept separate from commercial terms. FDA guidance describes this separation for contract manufacturing, where the quality agreement covers CGMP responsibilities and the supply agreement covers price and volume. For CROs, the transfer of sponsor obligations is written into the work order.

5.4 Oversight

Program owners review each tier 1 partner at least quarterly using four measures: deliverable timeliness, review outcomes, quality events and invoice accuracy. Reviews are recorded on the oversight form in the eQMS. Oversight includes work a partner subcontracts; ICH E6(R3) expects sponsors to oversee important transferred activities, including subcontracted ones, and to have access to providers' SOPs and performance metrics.

5.5 Audits and requalification

The Head of Quality approves an annual risk-based audit program each January. It lists tier 1 partners due for routine audit, planned for-cause audits and internal audits. A partner is requalified after each routine audit; a critical finding suspends new GxP work orders until QA accepts the response.

5.6 Exit

Before a relationship ends, the program owner confirms that all records, data and metadata have been transferred or archived, that retained samples are accounted for, and that the partner has

been removed from system access lists. Outsourcing & Procurement marks the partner closed in the register.

6. Records

Qualification files, audit reports, quality agreements and oversight forms are kept in the eQMS. The partner register is maintained by Outsourcing & Procurement.

7. Revision history

Rev. 4.0, January 2026 – moved to the eQMS; tier definitions revised; qualified-with-conditions status added; quarterly oversight measures defined.

Rev. 3.0, June 2024 – forms VQ-01 and VQ-02 retired; audit frequency for tier 2 changed from 24 to 36 months.